DATEN & FAKTEN

MOST IMPORTANT:

1. **Mehr als 50% der Menschen in Deutschland haben weder Patientenverfügung noch Vorsorgevollmacht Quelle: Deutscher Alterssurvey 2023 / bpb Sozialbericht 2024**
2. **421 physician queries in 2025**, versus more than 6.8 million registered documents
3. **No robust, directly measured German cost study was identified in the reviewed sources. Quelle Report Bundesnotarkammer 2025**

Änderungen 1. Oktober 2026 Bundesnotarkammer:

Seit dem 1. Oktober 2026 können im Zentralen Vorsorgeregister (ZVR) erstmals elektronische Abschriften (PDF-Scans) von Vorsorgedokumenten wie Patientenverfügungen oder Vorsorgevollmachten hinterlegt werden, anstatt wie bisher nur die reine Information über deren Existenz. Privatpersonen können den Upload nicht selbst durchführen, sondern müssen dafür eine verifizierte Institution – wie ein Notariat, eine Anwaltskanzlei oder eine Betreuungsbehörde – beauftragen. Im medizinischen Notfall oder bei anstehenden Betreuungsverfahren können berechtigte Ärztinnen und Ärzte sowie Gerichte dadurch den genauen Wortlaut der Verfügung sofort digital einsehen.

ePA

Unter vier Prozent nutzen elektronische Patientenakte aktiv Quelle: Deutsches Ärzteblatt 7. Januar 2026

In Deutschland haben rund 73 Millionen gesetzlich Versicherte automatisch eine elektronische Patientenakte (ePA) angelegt bekommen Quelle Gematik 2026

Germany has reasonably strong evidence that **many people lack advance care planning documents** and that, in emergency care, documents that exist are **often not physically available or not sufficiently actionable**. By contrast, there is **no robust German evidence proving that a missing document directly causes higher hospital costs**; economic claims should therefore be framed as hypotheses, not proven savings.

A. Problem prevalence

Measure	Strongest German figure	Source, population and method	Evidence quality / caveat
Adults aged 50+ with a **Patientenverfügung**	**44.8%**	Deutscher Alterssurvey (DEAS) 2020/21; $n = 4{,}185$people aged 50–90; nationally representative; self-report. rki	High for prevalence; self-reported possession, not verified document existence.
Women aged 50+ with a Patientenverfügung	**50.1%**	DEAS 2020/21. rki	Representative; self-report.
Men aged 50+ with a Patientenverfügung	**39.2%**	DEAS 2020/21. rki	Representative; self-report.

Measure	Strongest German figure	Source, population and method	Evidence quality / caveat
Age gradient, women	31.7% at 50–64; 61.5% at 65–74; 76.5% at 75+	DEAS 2020/21. rki	Strong age gradient.
Age gradient, men	28.9% at 50–64; 39.6% at 65–74; 68.5% at 75+	DEAS 2020/21. rki	Strong age gradient.
Education difference	No statistically significant difference	DEAS 2020/21. rki	Useful counterpoint to assumptions that uptake is primarily education-driven.
People with a Patientenverfügung who also have a **Vorsorgevollmacht**	**84.9%**	DEAS 2020/21, among people reporting a Patientenverfügung. rki	Conditional percentage; not the share of all adults with both.
Adults aged 45–90 with a Patientenverfügung	**45%** (44.6% unrounded)	DEAS 2023; bpb report; weighted survey data. bpb+1	Newer DEAS wave; self-report.
Adults aged 45–90 with a Vorsorgevollmacht	**41%** (41.4% unrounded)	DEAS 2023. bpb+1	Self-report; not necessarily medically specific or immediately usable.
Adults aged 45–90 with a Betreuungsverfügung	**29%** (29.0% unrounded)	DEAS 2023. bpb+1	Distinct legal document from a Patientenverfügung.
Adults with **all three** documents	**26%**	DEAS 2023; people aged 45–90. bpb	Strong headline gap, but “all three” includes a Betreuungsverfügung.
Adults with **none** of the three documents	**About 50%**	DEAS 2023; “every second person.” bpb	Best broad prevalence-gap figure for the 45–90 population.
Adults aged 16+ with a Patientenverfügung	**35%**	Deutsche Bank / survey of 1,086 people, fieldwork Aug–Sep 2024. db	Lower evidence tier than DEAS; useful for all-adult context.

Measure	Strongest German figure	Source, population and method	Evidence quality / caveat
Adults aged 16+ with a Vorsorgevollmacht	**31%**	Same survey. db	Self-report; commercial sponsor, so use as secondary support.

Pitch-ready framing: Among Germans aged 45–90, roughly **one in two has none** of the three core precautionary documents; among those aged 50+, fewer than half report having a Patientenverfügung.rki+1

B. Availability gap

The key distinction is: **a document may exist but still be unavailable, unclear, or not applicable to the acute clinical situation.**

Measure	Evidence	Setting / country	Strength and caveat
Emergency-department patients reporting any advance directive	**19.9–27.8%**	Systematic review of ED studies, published 2024; mostly international. pmc.ncbi.nlm.nih+1	Systematic review; heterogeneous studies; not Germany-specific.
Advance directive actually available at ED presentation, general adult patients	**0.3–6.8%**	Same systematic review. pmc.ncbi.nlm.nih+1	Strong evidence of an availability gap, but international transferability must be labelled.
Existing advance directives available when needed	In five studies, **11.6% or less**	Same systematic review. pmc.ncbi.nlm.nih	Shows possession does not equal clinical availability.
German ED study: patients with a Patientenverfügung	**One third had none; of 138 patients who said they had one, only 16 could present it**	Herne, Germany; two-month ED survey; Dtsch Med Wochenschr 2015. faz	Older, single-centre German study; directly relevant but limited generalizability.
German emergency clinicians with access to a pre-existing document in their last relevant emergency	**18%** of $n = 784$surveyed professionals; documents were also often incomplete or unclear	Germany; recent national clinician survey cited in a 2026 emergency-medicine article. link.springer	Clinician-reported and likely from a recent survey; useful for the availability/quality gap, but not a patient-level prevalence estimate.

Measure	Evidence	Setting / country	Strength and caveat
Documents often unclear, unspecific, or not applicable	Qualitative/systematic-review finding; German legal framework requires assessing applicability	Germany and international. link.springer+1	Directionally strong; no single reliable German percentage for "unclear documents."
No authorized representative identifiable or reachable	**No reliable German national quantitative estimate identified**	—	Do not use a number here in a pitch.

Important legal context: If no Patientenverfügung exists, or its instructions do not fit the current situation, a representative must establish the patient's **mutmaßlicher Wille**—their presumed wishes—based on prior statements, values, and beliefs.gesetze-im-internet+2

C. ZVR registrations and queries

The Zentrales Vorsorgeregister (ZVR) records whether precautionary documents exist and where they are located; registration is voluntary, so ZVR counts **understate** total document possession.rki+1

ZVR indicator	2024	2025	Source
New registrations	368,930	**391,842**	Bundesnotarkammer annual reports. bnotk+1
Total registered Vorsorgeverfügungen at year end	6,417,603	**6,817,992**	Bundesnotarkammer annual reports. vorsorgeregister
Changes / revocations	63,675	**66,870**	Bundesnotarkammer annual report. vorsorgeregister
Queries by guardianship courts	168,727	**162,673**	Bundesnotarkammer annual reports. bnotk+1
Queries by physicians	312	**421**	Bundesnotarkammer annual reports. bnotk+1
Queries with at least one matching registration	8,979 (5.3%)	**9,879 (6.0%)**	Bundesnotarkammer annual reports. bnotk+1
New registrations with no trusted-contact details	2024 not stated in retrieved text	**6.8%**	2025 annual report. vorsorgeregister

Critical interpretation: Physician queries are extremely low relative to registrations: **421 physician queries in 2025**, versus more than 6.8 million registered documents. This is a powerful adoption/access argument—but it does **not** prove that few documents are used clinically, because physicians may obtain documents through relatives, paper records, hospital systems, or direct presentation rather than ZVR.vorsorgeregister

The ZVR does **not** publish a breakdown of hospital versus individual-physician queries in the retrieved official reports; do not invent one.vorsorgeregister

D. Clinical and operational impact

Outcome	Evidence	Causality	Relevance
Need to determine presumed wishes when no valid/applicable directive exists	German law explicitly requires this process. gesetze-im-internet+1	Legal requirement, not a measured workload effect	Highly relevant; establishes the operational pathway.
Emergency clinicians report difficulty accessing documents; accessed documents often incomplete or unclear	German national clinician survey, $n = 784$: 18% had access in the last relevant emergency. link.springer	Association / reported experience; no causal cost estimate	Strong problem-validation evidence for Germany.
Low availability in EDs despite document existence	Systematic review: availability generally 0.3–6.8% among adult ED patients; in five studies, ≤11.6% of existing documents were available. pmc.ncbi.nlm.nih+1	Association; no causal claim	International; use as transferable evidence of the access gap.
ICU patients with a proxy / directive	In one German ICU study of 1,000 end-of-life decisions, only 44% had a proxy and 27% had a Patientenverfügung. pmc.ncbi.nlm.nih	Observational	German, hospital-based; shows documentation gaps among high-acuity patients.
Palliative/hospital patients with documents	German study of 294 palliative-care patients: 33% had an advance directive; 45% had a healthcare proxy. pmc.ncbi.nlm.nih	Observational	Relevant clinical subpopulation; not population-representative.
Additional physician/nursing time, delays, avoidable ICU admissions, or conflicts	**No sufficiently robust German study quantifying these outcomes specifically attributable to missing/inaccessible documents was identified in the reviewed evidence.**	Not established	State this explicitly; avoid causal claims.

A widely cited German ICU analysis reportedly found that the presence of a Patientenverfügung did **not** materially change organ-support use or ICU duration in 477 treatment cases; this further warns against claiming that documents automatically reduce intensive-care utilization.deutschlandfunkkultur

E. Economic impact

Type of evidence	What can be said	German transferability
Directly measured German costs of missing/inaccessible documents	**No robust, directly measured German cost study was identified in the reviewed sources.**	Not available
Staff time / coordination costs	No reliable German monetary valuation specifically tied to missing or unclear documents was identified.	Not available
Hospital length of stay / ICU costs	No German study reviewed establishes that missing documents cause higher LOS or ICU costs. One German ICU analysis found no measurable effect of directive presence on ICU duration or organ-support use. deutschlandfunkkultur	Do not claim savings
Structured ACP intervention economics	No German cost-effectiveness trial was identified in the reviewed results; international evidence would need separate sourcing and explicit transferability caveats.	Not established for Germany

Pitch-safe economic statement: The evidence supports a **documented operational and autonomy problem**, not yet a proven cost-saving business case. A credible pitch should say: "If inaccessible or unclear documents trigger avoidable clarification, coordination, or unwanted treatment, there may be economic value—but this remains unproven in Germany and requires measurement."

F. Five strongest pitch figures

1. **"About half of Germans aged 45–90 have none of the three core precautionary documents."**
    - Figure: ~50% have neither a Patientenverfügung, Vorsorgevollmacht, nor Betreuungsverfügung.bpb
    - Source: DEAS 2023, reported by bpb in 2024.
    - Caveat: Self-reported; age-restricted population.
2. **"Fewer than half of people aged 50+ have a Patientenverfügung."**
    - Figure: 44.8%; women 50.1%, men 39.2%.rki
    - Source: RKI / DEAS 2020/21, $n = 4{,}185$.
    - Caveat: Self-reported possession, not verified availability.
3. **"Even when an advance directive exists, it is often not available in an emergency."**
    - Figure: In general adult ED populations, availability ranged from 0.3% to 6.8%; among studies measuring both existence and availability, five reported availability of 11.6% or less.pmc.ncbi.nlm.nih+1

- Source: 2024 systematic review.
- Caveat: International evidence; do not present as a German national rate.

4. **"German emergency clinicians report access in only 18% of recent relevant emergencies—and the documents they access are often incomplete or unclear."**
   - Figure: 18% of $n = 784$professionals.link.springer
   - Source: German national clinician survey cited in a 2026 emergency-medicine article.
   - Caveat: Clinician-reported; not a patient-level audit.
5. **"Physicians queried Germany's official ZVR only 421 times in 2025, despite 6.82 million registered documents."**
   - Figure: 421 physician queries; 6,817,992 registered Vorsorgeverfügungen at year-end 2025.vorsorgeregister
   - Source: Bundesnotarkammer ZVR annual report 2025.
   - Caveat: Low ZVR queries do not prove low clinical use of documents obtained through other channels.

G. Evidence gaps

- **No German national figure** separates "no document," "document exists but cannot be found," and "document exists but is unclear/not applicable."link.springer+1
- **No reliable German national estimate** exists for how often no authorized representative can be identified or reached.
- **No robust German health-economic study** directly measures costs caused by missing, inaccessible, or unclear documents.
- ZVR query statistics show registry interaction, not clinical implementation; physician queries are published, but hospital-specific query counts are not.vorsorgeregister
- Most availability evidence comes from emergency departments and is partly international; results cannot be directly extrapolated to German hospitals without German replication.pmc.ncbi.nlm.nih+1
- Possession surveys are self-reported and do not verify whether documents are valid, current, specific, or accessible at the point of care